Fiduciary Action Checklist

Every deadline, contract term, and decision the new PBM rule puts on your desk

EDITION 2026.2 · NOVEMBER 2026 · DOL FINAL RULE 29 CFR 2550.408B-22 AND CAA 2026

AUDIENCE	Plan sponsors, plan fiduciaries, and the brokers, consultants, and attorneys who advise them
PURPOSE	What to do, by when, under the DOL final PBM rule and CAA 2026: who owes you disclosures, which contract terms to elect and strike, and how to act when disclosures arrive
COMPANION	The free PBM Rule Navigator applies this checklist to your plan in two minutes. Contract X-Ray tests your actual contract
ONLINE	nautilushealth.org/PBM · contractxray.com

Disclosure shows you the money. Acting on it is the fiduciary duty.

On October 29, 2026, the Department of Labor released its final PBM fee disclosure rule. For plan years beginning on or after July 1, 2027, PBMs serving large self-insured ERISA plans, and any other vendor that performs pharmacy benefit services for them, must disclose, in dollars and drug by drug, what they and their affiliates earn: rebates kept, spread on every NDC, pharmacy clawbacks, termination fees, and the net price the plan pays.

They must disclose it before you sign or renew, again every six months, and open their records to an auditor you choose.

The rule shifts the burden of proof. A PBM that fails to disclose is in a prohibited transaction. A fiduciary who receives the disclosures and does nothing with them has a documentation problem of its own. Every dollar retained upstream is a dollar your employees and their families pay through premiums and cost sharing.

DATE	WHAT HAPPENS
February 3, 2026	Consolidated Appropriations Act (CAA) of 2026 disclosure duty under ERISA 408(b)(2)(B) took effect. DOL grants good-faith enforcement relief until the rule applies
January 4, 2027	Final rule becomes effective
90 days before your first renewal on or after July 1, 2027	Initial disclosures due from your PBM (October 2027 for a January 1, 2028 renewal). For a new contract the rule says only “reasonably in advance,” so set a date in the contract
Plan years beginning on or after July 1, 2027	Rule applies. For calendar-year plans: January 1, 2028
Plan years beginning on or after August 3, 2028	CAA 2026 100% rebate pass-through and new semiannual PBM reports apply

Items marked pending counsel confirmation rest on our reading and await legal review. To apply this checklist to your own plan, use the free PBM Rule Navigator at nautilushealth.org/PBM.

STEP 0

Know Which Rules Reach Your Plan

The answer turns on three facts: whether ERISA governs the plan, whether it's self-insured, and whether it has 100 or more employees or participants. Level-funded plans count as self-insured. Self-insured dental and vision plans are covered plans too, so check whether any of their vendors provide pharmacy-related services. Two exclusions are narrower than they look. A plan counts as fully insured only if all of its benefits are insured, so a self-funded pharmacy carve-out next to insured medical is covered. And plans sponsored by a drug manufacturer, distributor, wholesaler, PBM, or rebate aggregator, or an affiliate, fall outside the detailed rule.

PLAN TYPE	DOL FINAL RULE	CAA 2026 PASS-THROUGH (2028)	CAA 2026 PBM REPORTS (2028)
ERISA, self-insured or level-funded, 100+	Applies in full	Applies	Detailed semiannual report
ERISA, self-insured, under 100	General statutory disclosures only	Applies	Summary report
ERISA, fully insured (all benefits, including drugs)	Detailed rule doesn't apply. General statutory disclosures still required	Applies through the insurer	Summary report; large plans may opt in to detailed
Non-federal governmental (county, city, school district)	Does not apply	Does not apply	Applies
Church plan (non-electing)	Does not apply	Does not apply	Applies

Two counting rules matter. Single-employer plans count employees using the ACA employer aggregation rules. Multiemployer plans and MEWAs count participants, averaged over the prior plan year.

Governmental and church plans still carry fiduciary duties under state law and plan documents. For them, every item below becomes a contract ask: require the PBM to make these disclosures and accept these audit terms as if ERISA applied.

Who owes you disclosures. The rule follows the service, not the label. Any vendor that contracts with your plan and expects $1,000 or more is covered if it provides any of these: rebate negotiation or aggregation, a formulary, a pharmacy network (including specialty, infusion, and outpatient pharmacies), or drug utilization management such as prior authorization, step therapy, adherence programs, and generic or biosimilar substitution. Drugs paid under the medical benefit count. So most medical TPAs are in scope, along with utilization management vendors, specialty and site-of-care programs, and alternative funding programs.

A TPA that contracts to provide pharmacy services owes you the full disclosures even when a PBM does the work and you have no contract with that PBM. The TPA must get the PBM's data, disclose what the PBM keeps, and give your auditor the PBM's records. If the PBM fails, the plan names the TPA in its notice to DOL, and DOL declined to give TPAs any relief. The same applies to level-funded arrangers and to the operator of a MEWA made up of separate employer plans.

- [ ] Confirm ERISA status, funding arrangement, and headcount for the current year
- [ ] Identify who signed the pharmacy contract. If a TPA, carrier, or level-funded arranger contracts for pharmacy services, that entity owes you the disclosures, even when another company runs the PBM. When a TPA relies on a PBM to prepare its disclosures, require the TPA's contract with that PBM to deliver the data in time for the TPA's own deadlines
- [ ] Ask your broker or consultant in writing whether it is affiliated with your PBM. Affiliated brokers and consultants must disclose under the rule
- [ ] List every vendor that touches a drug: PBM, medical TPA, utilization management, specialty and infusion programs, alternative funding, GLP-1 or condition programs, and rebate aggregators. Note which ones contract with the plan
- [ ] Ask your TPA in writing what rebates, administrative fees, and other payments it or its partners receive on drugs paid under the medical benefit

NOW THROUGH MID-2027

Know Your Contract Before Your PBM Explains It

The grace period is a head start, not a pause. DOL confirmed the statutory disclosure duty has applied since February 3, 2026, and it protects fiduciaries who act in good faith until the rule applies. Good faith leaves a paper trail.

1. Pull the complete contract. The master agreement, every pricing and rebate exhibit, every amendment, performance guarantees, the business associate agreement, and the renewal, notice, and termination terms. Most sponsors have never assembled the full set in one place. If your PBM or TPA holds the only copy, request it in writing.
2. Run a Contract X-Ray analysis. It evaluates the contract against Fiduciary-Aligned PBM Contract Standards organized into 10 categories, from rebate pass-through and audit rights to data ownership and clean exit, and produces a Negotiation Report: the specific terms to request, the model language to propose, and the fiduciary rationale for each. That report becomes your renewal agenda and part of your documented process.
3. Request statutory disclosures now from your PBM, TPA, broker, and any other provider paid $1,000 or more. ERISA 408(b)(2)(B) has required them since February 3, 2026. Keep the request and the response. For your TPA, ask for the items DOL lists: administrative and per-employee-per-month fees, 340B-related income, provider and network compensation, affiliate and vertically integrated compensation, data monetization, and payment and recovery compensation.
4. Document a reasonableness review of the current arrangement, using the disclosures and the Contract X-Ray results. Minutes, not memory.

5. Build a renewal calendar. List every pharmacy-related contract, its renewal date, and its auto-renewal notice window. Work backward: the Negotiation Report should be in hand before the PBM's 90-day disclosure window opens.
6. Name the responsible plan fiduciary. The rule sends disclosures to the fiduciary with authority to sign, extend, or renew. Confirm who that is, and update the committee charter so disclosure review and audit decisions have an owner.

- [ ] Complete contract assembled and stored with the plan's fiduciary records
- [ ] Contract X-Ray analysis and Negotiation Report received
- [ ] Written disclosure requests sent to PBM, TPA, and broker
- [ ] Reasonableness review documented in committee minutes
- [ ] Renewal calendar built, with auto-renewal notice dates
- [ ] Responsible plan fiduciary named; committee charter updated
- [ ] Multi-year contract running past July 1, 2027 without a renewal: ask counsel how the rule applies (*pending counsel confirmation*)

BEFORE YOUR NEXT RENEWAL

Negotiate the Terms the Rule Leaves Open

The rule sets a floor and leaves several dials to you. Set them in the contract, because whatever you don't elect defaults to the PBM's preference.

Terms to elect

The rule permits each of these; none happens unless you ask.

ASK	DEFAULT IF YOU SAY NOTHING	SOURCE
Quarterly disclosure reports	Semiannual	2550.408b-22(e)(2), (g)
Materiality threshold below 5% for formulary-change notices	5% of manufacturer payments	(e)(6)(iii)
Overage explanation threshold below 5%	5% over estimate	(g)(4)
Machine-readable disclosure files within 30 days of request	"Reasonable time," undefined	(k)(3)
Audit "year" defined as the plan year	Left to the parties	(j)(1)
Initial disclosures more than 90 days before renewal, and by a fixed date before signing a new contract	90 days	(e)

Terms to strike

For covered plans, each of these makes the arrangement unreasonable under ERISA 408(b)(2) once the rule applies. Contract X-Ray flags them in the Negotiation Report.

CONTRACT TERM	WHY IT FAILS	SOURCE
PBM approval of, or limits on, your choice of auditor	Plan selects the auditor	(j)(2)
Audits less than once a year	At least annual, at plan request	(j)(1)
Limits on audit period, location, or number of records	No PBM restrictions	(j)(6)
Upstream rebate and aggregator contracts walled off from audit	PBM must produce them, plus affiliate and subcontractor records, under reasonable confidentiality	(j)(3), (j)(7)
Fees charged to the plan for producing audit data	PBM bears the cost of providing information	(j)(4)
Audit data due later than 60 days after request	60-day outer limit	(j)(5)
Confidentiality clauses barring you from sharing the contract or disclosures with consultants, auditors, or competing bidders	PBM may only require reasonable confidentiality on redisclosure; regulators and counsel always allowed	(k)(4)
Early-termination fees beyond the PBM's actual losses, or no exit on reasonably short notice	DOL: no arrangement is reasonable unless the plan can exit without penalty on reasonably short notice. The PBM may recoup only reasonable amounts for actual losses	Preamble p. 116; 29 CFR 2550.408b-2(c)(3); (e)(6)(iii)
Mandatory statistical sampling in audits	DOL declined to permit sampling expressly (*pending counsel confirmation*)	(j)(6)

Terms prudence calls for

The rule is silent on these. Ask anyway.

- [ ] No-fee termination right if the PBM fails to cure a disclosure or audit failure within 90 days
- [ ] All rebates earned through the termination date paid in full
- [ ] Plan data returned in usable form within 30 days of termination, at no charge
- [ ] PBM indemnifies the plan for losses caused by inaccurate disclosures
- [ ] PBM represents in the contract that every required disclosure is complete and accurate
- [ ] Audit rights cover contract compliance and permit extrapolation, beyond verifying the disclosures

- [ ] A “higher of” clause: a broader regulatory definition of remuneration or audit scope applies automatically; a narrower one leaves the contract intact
- [ ] Every RFP response discloses the bidder’s full economics in the rule’s disclosure categories, so competing offers compare line by line

WHEN DISCLOSURES ARRIVE

Read Them, Test Them, Act on Them

Disclosure is the raw material. The fiduciary act is what you do next, and DOL expects to see it written down. Drug-level disclosures also make it easy for plaintiffs’ lawyers to benchmark your plan, so that written record is your best defense.

Review each initial disclosure against the contract and your Contract X-Ray results. A contract promising 100% pass-through next to a disclosure showing retained manufacturer payments is a finding, not a footnote. Disclosure isn’t permission. If the contract requires an amount to be passed through, the PBM still owes it, even after the disclosure labels it retained.

- [ ] Disclosure arrived at least 90 days before renewal, or by the date your contract sets
- [ ] Every required element is present: services; direct compensation by service; manufacturer payments per drug, passed through and retained; spread per NDC and by channel; affiliated pharmacy steering; pharmacy clawbacks; price protection; termination compensation; other compensation and who pays it; formulary incentives and omitted lower-cost alternatives; net price per fill; fiduciary status; audit rights
- [ ] Amounts stated in dollars. Estimates appear only where an amount isn’t reasonably ascertainable, and each rests on the contract’s terms or historical experience
- [ ] Retained amounts reconciled against the contract’s pass-through and fee terms
- [ ] Drugs with retained manufacturer payments checked against the therapeutically equivalent alternatives the PBM left off the formulary
- [ ] “Other compensation” itemized, including revenue from copay maximizer, accumulator, and alternative funding programs, float, technology and data fees, shared savings, and rebates and fees on drugs paid under the medical benefit
- [ ] Reasonableness determination recorded in committee minutes before signing

Track the semiannual reports. Each is due within 30 days after the period closes. Any category running 5% or more over the estimate, or your lower threshold, requires a written explanation of the overage.

Decide on an audit every year, and record the decision. The plan pays the auditor and the PBM pays to produce the data. DOL says an audit isn’t required absent red flags, but the choice to skip one must be a prudent, documented choice. Budget for it, choose an independent auditor, and send the request in writing. The PBM must acknowledge within 10 business days and produce within 60 calendar days, absent extraordinary circumstances.

Follow the relief protocol the moment you find a failure. A missing disclosure or a stonewalled audit is the PBM's prohibited transaction. It becomes yours too unless you act on these steps. Relief is available when you did not know of the failure and reasonably believed the disclosures were made.

1 · REQUEST
Send the PBM a written request to correct the failure as soon as you discover it.

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2 · CURE WINDOW
If the PBM cures within 90 calendar days, relief is preserved. Keep monitoring.

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3 · NOTIFY DOL
If it refuses or misses the 90 days, notify DOL within 30 days of the refusal or of day 90, whichever comes first.

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4 · DECIDE
Decide whether to terminate or continue the contract under your duty of prudence, and document the basis.

The decision record should name the alternatives you weighed and how you'll prevent a repeat. DOL says it should not be a check-the-box exercise. Keep it with the plan's fiduciary records. (2550.408b-22(n))

A PBM or other covered provider that corrects a good-faith disclosure error within 30 days of learning of it hasn't broken the rule (2550.408b-22(l)). The protocol protects you, not the PBM, which stays exposed to civil penalties under ERISA 502(i). And termination is a judgment call, not a requirement: document why you stayed or left.

AUGUST 2028

Sign 2027 Contracts for the Law Arriving Next

When the 2028 requirements reach your contract. Two things have to be true: your contract is signed, renewed, or extended, and it covers a plan year that starts on or after August 3, 2028. For a calendar-year plan, the first such plan year starts January 1, 2029. Your contract's end date doesn't matter on its own. What matters is whether the contract covers a plan year that starts on or after August 3, 2028.

SCENARIO (CALENDAR-YEAR PLAN UNLESS NOTED)	DO THE 2028 REQUIREMENTS APPLY DURING THIS CONTRACT?
You sign tomorrow. The contract ends August 4, 2028.	No. Your 2028 plan year started January 1, 2028, before the cutoff. The requirements arrive with your next contract or renewal.
You sign tomorrow. The contract runs through December 31, 2028.	No. Same reason: it never covers a plan year starting after August 3, 2028.
You sign in 2027 for three years, through December 31, 2029.	Unsettled. The contract covers the 2029 plan year, which starts after the cutoff. One reading says the requirements apply for 2029; another says they wait for your next renewal. *Pending counsel confirmation*. Write them into the contract for 2029 now.
The contract auto-renews on January 1, 2029.	Yes, under the better reading. An automatic renewal for the 2029 plan year is a renewal. Some practitioners treat evergreen terms differently, so put the answer in writing.
You amend pricing mid-term in 2029.	Unsettled. The statute doesn't say whether an amendment counts as an extension. Treat any amendment as a renewal in the contract language.

SCENARIO (CALENDAR-YEAR PLAN UNLESS NOTED)	DO THE 2028 REQUIREMENTS APPLY DURING THIS CONTRACT?
Your plan year starts July 1.	First covered plan year: July 1, 2029. July 1, 2028 is before the cutoff.
Your plan year starts September 1.	First covered plan year: September 1, 2028.
You're a county, city, school district, or church plan.	The pass-through does not apply by statute; it amends ERISA only. The reporting requirements do apply. Contract for the pass-through anyway.

The practical answer: any contract you sign from now on that runs past mid-2028 should require the PBM to comply with the CAA 2026 PBM requirements starting with your first plan year that begins on or after August 3, 2028, regardless of contract cycle. It should also treat any renewal, extension, auto-renewal, or amendment as a renewal. Then the open questions don't matter, because the contract has already answered them.

The DOL disclosure rule runs on its own clock: plan years beginning on or after July 1, 2027, or January 1, 2028 for calendar-year plans.

What the statute requires of PBMs serving ERISA plans:

- [ ] Remit 100% of rebates, fees, alternative discounts, and other remuneration from manufacturers, rebate aggregators, and affiliated GPOs, quarterly, within 90 days of quarter end (ERISA 408(b)(2)(C))
- [ ] Rebate aggregators and PBM-affiliated GPOs must remit to the PBM within 45 days of quarter end
- [ ] Return underpayments within 90 days of notice
- [ ] Open remittance records and rebate contracts to audit at least once per plan year, by an auditor the plan selects and the PBM does not pay
- [ ] Keep "bona fide service fees" only if they are transparent and quantifiable. DOL has not yet defined the term
- [ ] These duties fall on whoever provides pharmacy benefit services, including carriers, TPAs, and rebate aggregators, whatever they call themselves

What the statute requires for all group health plans, governmental and church plans included (ERISA 726, PHSA 2799A-11, IRC 9826):

- [ ] Semiannual PBM reports, quarterly on request, in plain language and machine-readable format. Plans with 100 or more get drug-level and therapeutic-class detail, including spread on every claim
- [ ] Disclosure of what the PBM pays brokers and consultants to win or keep your business, with the recipient named
- [ ] An annual notice from the plan to participants about the PBM reporting requirement
- [ ] Civil penalties for PBMs and others that fail to report: up to $10,000 a day, and up to $100,000 for each item of knowingly false information

Spread pricing is reported under the statute, not banned. If you want it gone, the contract has to say so.

- [ ] Require compliance with the 2028 provisions by a fixed date, the first plan year beginning on or after August 3, 2028, regardless of contract cycle
- [ ] Define any amendment, extension, or auto-renewal as a renewal for CAA 2026 purposes
- [ ] Define remuneration broadly: any value, in any form, from any manufacturer, aggregator, or affiliate, including margin retained between plan payment and pharmacy reimbursement
- [ ] Require pass-through pricing so no spread exists
- [ ] Define a manufacturer fee as bona fide only if it is flat, fair market value, for an identified service, unrelated to price or volume, and itemized by payer
- [ ] Governmental and church plans: require the PBM to meet the 408(b)(2)(C) pass-through terms as if ERISA applied

The 2028 requirements are enacted law under CAA 2026 Sections 6701 and 6702. Only the interpretive questions listed at the end of this checklist await counsel confirmation.

WATCH LIST

Questions Still Open

DOL left several questions for later. Each could change what you ask for, so revisit this list at every renewal.

OPEN QUESTION	WHERE IT STANDS	WHAT TO DO NOW
Fiduciary safe harbor for a structured PBM selection process	DOL "strongly agrees" and is considering separate rulemaking under ERISA 404(a)	Run and document a structured evaluation now. It's the process any safe harbor would reward
Definition of "bona fide service fee"	DOL called it premature; future rulemaking or guidance	Contract for the strict definition in the 2028 section
Standard disclosure template and machine-readable format	DOL is considering a template after publication	Elect machine-readable delivery and a 30-day deadline
Detailed disclosure rules for TPAs' other services	Separate future project for services other than pharmacy benefit services. A TPA that provides pharmacy services is already covered (see Step 0). DOL expects good-faith statutory disclosure now, naming admin fees, 340B income, network and affiliate compensation, and data monetization	Request those items from your TPA today
Whether plan claims data is a plan asset	Not answered	Secure data ownership and access by contract

OPEN QUESTION	WHERE IT STANDS	WHAT TO DO NOW
Claims data outside an audit	DOL declined to require it	Negotiate standing access to claims-level data
Legal challenge to the rule	The PBM trade group says DOL may have exceeded its authority; a lawsuit could delay or stay the rule. No suit had been filed as of November 6, 2026	Act anyway. The statutory disclosure duty has applied since February 3, 2026, and these contract terms protect the plan either way
How state PBM laws interact with the rule	DOL declined to say whether the rule preempts state law, calling it "an interpretive matter"	If your plan covers employees in several states, ask counsel how state PBM laws apply alongside the rule and your contract

Items pending counsel confirmation (the CAA 2026 requirements and dates themselves are enacted law):

- [ ] Treatment of multi-year contracts that don't renew before July 1, 2027
- [ ] Whether a contract requiring statistical sampling in audits conflicts with the rule
- [ ] Which contract events trigger CAA 2026, and what "bona fide service fee" means

Put the Checklist to Work

STEP 1 · YOU ARE HERE

Fiduciary Action Checklist

Every deadline, contract term, and decision the rule puts on your desk.

STEP 2 · FREE

PBM Rule Navigator

Your plan's deadlines and action plan in two minutes. Ask anything; every answer cites the rule.
nautilushealth.org/PBM

STEP 3 · FREE

Contract X-Ray Quick Look

Test your actual PBM contract against fiduciary-aligned standards.
contractxray.com

STEP 4

Full Assessment

A Negotiation Report with the terms to request and the model language to propose.

Sources

Husch Blackwell: DOL Finalizes PBM Fee Disclosure Rules
Bloomberg Law: Trump Finalizes PBM Transparency Rule
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Federal Register: Final rule, 91 FR 69092 (November 2, 2026)
Sheppard Mullin: PBM Transparency Moves Forward
Troutman Pepper Locke: DOL Final Rule Expands PBM Transparency
PlanSponsor: Groom attorneys on PBM disclosure and litigation risk
Mayer Brown: DOL Issues Final PBM Transparency Rule
Morgan Lewis: DOL Finalizes PBM Transparency Rule
Mintz: From Proposal to Final Rule

This checklist explains regulatory requirements for plan sponsors and their advisors. It is not legal advice. Confirm plan-specific questions with ERISA counsel.